



Fiona Stanley Fremantle Hospitals Group

Procedure

Non-prescribed substances: Management in the Inpatient Setting

Reference #: FSFH-MENH-PRO-0015

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Mental Health Service	Medical, Nursing, Allied Health

1. Introduction

It is recognised the use, possession and / or distribution of alcohol and / or other drugs and / or non-prescribed medications by a mental health inpatient or visitor may negatively impact the treatment and recovery of the patient, other patients and the therapeutic milieu on the ward. This document outlines the procedure for preventing and managing the presence of non-prescribed substances on the inpatient wards.

2. Terminology

Alcohol	Alcohol is a central nervous system depressant/sedative drug. Even in small doses alcohol can significantly impede motor and cognitive function depending on the dose and individual susceptibility.
Alcohol Smoking Substance Involvement Screening Tool (ASSIST-Lite)	A short risk identification tool for use with adults (aged 18 years or over) covering alcohol, tobacco, cannabis, stimulants, sedatives, opioids and other psychoactive substances including the use of medicines not as prescribed. The ASSIST-Lite allows for assessment of risk and formulation of action/management plans
Drug	Any substance (except for food and water) which, when taken into the body, alters the body's function either physically and/or psychologically. Drugs may be legal e.g. alcohol, prescription medication or illicit e.g. methamphetamine.
Intoxication	A condition that follows the administration of a psychoactive substance and results in disturbances in the level of consciousness, cognition, judgement, affect, or behaviour, or other psychophysiological functions and responses.

Urine drug screen (UDS)	A visual panel of immunoassays that can be used for the simultaneous qualitative detection of multiple drug metabolites in human urine. Widely accepted method for alcohol and other drug screening.
Store/storage	Removal of prohibited or unsafe items from the person or their belongings and stored at the FSFH site until such time as the item can be safely returned to the person.
Prohibited items	According to the Health Services (Conduct and Traffic) Regulations 2016 includes: • An alcoholic beverage • A prohibited drug as defined in the Misuse of Drugs Act 1981

3. Procedure

3.1. Prevention

- On admission patients will be:
 - provided with written information on mutual expectations.
 - screened for alcohol and drug use using the ASSIST-Lite
 - asked if they smoke and informed nicotine replacement therapy is available and recommended for patients; even those who use cannabis and mix tobacco with it.
 - asked if they would like referral to the FSH Alcohol and other Drug Service or FH Dual Diagnosis Service for ongoing interventions and support.
- On admission **and** prior to any leave patients (where appropriate) will be informed:
 - the use, possession, or distribution of alcohol and / or other drugs and / or medications not prescribed by the treating team are not allowed during the inpatient admission.
 - of the requirement for testing where there is a strong suspicion of alcohol / other drug use and/or signs of intoxication impacting the consumer's physical or mental state.
 - a room search will be conducted when there is suspicion/confirmation that the patient is in possession of illegal substances.
- Prior to leave there will be a discussion with the patient regarding harm reduction and potential strategies for managing cravings while on leave where appropriate.

3.2. Management.

- Report to the Shift Coordinator if a patient is suspected/known to have consumed alcohol/other drugs or medications not prescribed by the team.

- Complete a set of physiological observations and monitor the patient for signs or symptoms of use.
- Maintain the safety of the patient, other patients, visitors, and staff.
- The Shift Coordinator must inform the treating team doctor/Duty Medical Officer (DMO), Nurse Unit Manager (NUM)/Clinical Nurse Specialist (CNS)/ HOOT Clinical Nurse Manager immediately when informed by a staff member.
- The treating team doctor/DMO will review the patient. Based on medical review and risk assessments, the reviewing doctor will determine whether to withhold regular medications until the patient is stable and/or may decide to amend the patient's Treatment Support and Discharge Plan as required
- Establish the patient's level of intoxication by:
 - discussion with the patient around what and how much they have used and / or
 - UDS
 - breathalyser
- A brief intervention discussion should occur with the patient regarding harm reduction and psychoeducation in relation to their alcohol and or other drug use and its impact on their mental health recovery:
 - recommend referral to the FSH Alcohol and other Drug Service or FH Dual Diagnosis Service for ongoing interventions and support.
- The Shift Coordinator / DMO / treating team doctor will determine if close or continuous supportive observations are to be initiated. Refer to the FSFHG Patient Observation - Specialising and Close Observations.
- If the patient is assessed as medically compromised the DMO / treating team doctor will either:
 - liaise with the Emergency Department regarding transfer of FH /FSH-CH patients for further assessment / management.
 - arrange transfer to medical bed for FSH patients for further assessment / management.
- Activate a Code Blue if the patient's physical state is acutely compromised. Refer to the Code Blue – Medical Emergency Action Card.
- The NUM / CNS/ HOOT A/Hrs will inform Security if items need to be removed and stored.
- Document the incident, observations and assessments and management plan in the patient's medical record.
- Document the clinical incident in Datix CIMS
- Update alerts in PSOLIS

3.3. Confirmed possession.

- If possession of alcohol and /or other drugs and/ or medications not prescribed by the treating team is confirmed, the Shift Coordinator with another staff member and/or Security will request that the patient submits the identified substance.

Table 1: Confirmed possession actions.

Substance	Actions
Alcohol	• Discuss with the patient cravings, using, alcohol and mental health, medication interactions and harm reduction from a health perspective. • Request the patient remove the alcohol from the hospital grounds, either by themselves, or through a carer or personal support person • If the patient is unable or refuses to comply with removing the alcohol, Security will be requested to remove the alcohol from the clinical area. • Opened vessels will be disposed of appropriately following approval from the Nurse Unit Manager/HOOT CNM and unopened vessels will be stored as patient property. • Review the patient's leave status. • At discharge clinical staff will complete a risk assessment and use clinical judgement to determine whether to return alcohol to the patient/family and document outcome in the patient's medical record.
Illicit substances	• Discuss with the patient cravings, using, illicit substances and mental health, medication interactions and harm reduction from a health perspective. • The Shift Coordinator with another staff member and/or Security will request the patient hands over the illicit substance. • If the patient complies, the illicit substance is to be placed in an envelope and sealed. The following information must be recorded on the envelope: ○ Date and time (24-hour format) the illicit substance / non-prescribed medications was placed in the envelope. ○ Printed names and designations of staff members who placed the illicit substance in the envelope; and staff who attended at the time. ○ Signatures of each of the staff members whose names are recorded on the envelope. ○ Patient label. • The envelope is provided to security for storage. • If the patient refuses to hand over the illicit substance, staff are to observe the patient until Security arrive. • Staff will provide Security with the demographic details of the patient. • Security staff will contact WA Police. • Notify as a clinical incident via Datix CIMS. • Document the incident, observations and actions taken in the medical record.

	• Review the patients leave status
Medications not prescribed by treating team	• Discuss with the patient cravings, using, medications and mental health, medication interactions and harm reduction from a health perspective. • Report possession and/or distribution to the Shift Coordinator. • The Shift Coordinator will alert all staff on the ward and ensure staff understand they are not to approach the patient alone. • The patient is informed that the medications will be removed and the treating team will determine the medication regimen for the inpatient admission. • The non-prescribed medication, will be stored as per the FSFHG Medication Administration policy and procedure

3.4. Testing patients for alcohol or other drugs

- Patients may be requested to provide a routine UDS on admission to the ward.
- Testing may also be undertaken in situations where there is strong suspicion of alcohol / other drug use by a patient and where such use could impact on the patient's mental state, physical state or compromise the patient's treatment plan.
- Relevant medical staff must be involved in the decision-making process regarding testing the patient.
- Refer to the SMHS Testing mental health inpatients for alcohol and/or illicit substance use policy.
- The patient must be provided with an explanation of the reason for the testing, including potential outcomes; and this must be documented in the medical record.
- The patient's consent must be obtained prior to testing for alcohol or other drug use.
- If the patient refuses to be tested, the patient's behaviour, demeanour and actions will be documented and discussed with the treating team at the earliest point.
- The Shift Coordinator / treating team doctor / DMO will determine if close and continuous supportive observations are to be initiated.

3.5. Searching a consumer and / or their property

- Where staff hold a reasonable concern that a patient may be in possession of items considered a danger to themselves or others, a search of that patient may be undertaken.
 - Refer to the Refer to SMHS Patient search and identification of dangerous items procedure.

3.6. Decision to discharge

- The Shift Coordinator may consult with the treating team doctor / DMO regarding a decision to discharge patients/s in the possession of; and/or

responsible for the distribution of alcohol / other drugs / medications not prescribed by the treating team.

- The treating team doctor / DMO will consider as a minimum, the patients current:
 - Diagnosis
 - Mental state.
 - Capacity
 - Risk of harm to self, to others, from others
 - Treatment plan
- Consider best practice in the management of patients with ongoing features of psychosis in the context of substance use, including whether the patient meets the criteria for the Mental Health Act 2014 and requires detention.
- If a decision to discharge the patient is made this will be documented in the patients DMR including decision making rationale and appropriate safety planning.

4. Support on discharge.

- Wherever possible as part of discharge planning the patient will be referred to external service/s of their choice that may assist with providing support related to their alcohol and other drug use.
- A comprehensive discharge plan including outcomes of risk assessments, interventions, treatment plans and follow up arrangements must be documented in the patient's medical record and include relevant AOD use.
- The discharge plan will be made in collaboration with the patient and their family/significant other.

5. Visitors

- Any visitor to the mental health service found in possession of alcohol or prohibited/other substances, staff will:
 - Request the visitor of a mental health patient to self-search if indicated.
 - Request the visitor to leave the premises.
 - Contact security or manager of the clinical area if the visitor refuses to leave.
 - Security and senior staff will determine if WA Police are to be contacted.
 - Document outcomes in the patient's medical record including visitor restrictions need to be considered.

6. Compliance/Performance Monitoring

Compliance against this procedure will be monitored by the relevant NUM/CNS via routine monitoring of clinical incidents.

7. Related Policy Documents

Department of Health:

- [Alcohol and other drug withdrawal management policy \(MP0062/17\)](#)
- [Consent to Treatment Policy \(MP0175/22\)](#)
- [Clinical Incident Management Policy](#)

SMHS:

- [Alcohol and Prohibited Substances](#) (SMHS: 112)
- [Testing mental health inpatients for alcohol and/or illicit substance use](#) (SMHS: 151)
- [Patient search and identification of dangerous items](#)

FSFHG

- Alcohol and Other Drugs: management of addiction, toxicity and withdrawal. (FSFH-HW-POL-0063)
- Prevention and management of unpredictable behaviours (FSFH-MENH-POL-0026)
- Medication Administration (FSFH-HW-POL-0022)
- Medication Management of Agitation and Arousal in Mental Health Inpatients guideline (FSFH-MENH-GUI-0004)
- Patient observation-specialising and close observations (FSFH-MENH-POL-0023)
- FSH Code Blue - Medical Emergency
- FH Code Blue - Medical Emergency
- Visiting in mental health inpatient areas (FSFH-MENH-POL-0016)
- Discharge planning for mental health inpatients (FSFH-MENH-POL-0032)
- Clinical documentation (FSFH-HW-POL-0039)

8. Related Legislation

- [Mental Health Act 2014](#)
- [Health Services Act 2016](#)

9. Related Standards

NSQHS Standards:

- Clinical Governance

- Partnering with Consumers
- Medication Safety
- Comprehensive Care
- Communicating for safety

Chief Psychiatrists Standards for Clinical Care:

- Care Planning
- Consumer and carer involvement in individual care
- Risk assessment and management.

10. Authorisation

EXECUTIVE SPONSOR: Service Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	04/2023	Nurse Coordinator	Mental Health team	Service 5 Safety Quality and Risk Committee	12/2024
2	Feb 2025	Nurse Coordinator	Mental Health team	FSFH Clinical Governance Sub Committee	03/2028